

CORIFED TABLET

Composition:

Each tablet contains:

Triprolidine Hydrochloride	2.5mg
Pseudoephedrine Hydrochloride	60mg

Description:

A yellow round shaped tablet with score line on one side.

Pharmacodynamics:

Pseudoephedrine is an orally effective upper respiratory tract decongestant. It acts on alpha-adrenergic receptors in the mucosa of the respiratory tract, producing vasoconstriction. It shrinks swollen nasal mucous membrane; reduces tissue hyperemia, edema and nasal congestion; and increases nasal airway potency. Drainage of sinus secretions may be increased and obstructed eustachian ostia may be opened.

Triprolidine is an antihistamine (H1-receptor). It acts by competing with histamine for H1-receptor sites on effector cells. It prevents, but does not reverse, responses mediated by histamine alone. It also has anticholinergic actions and provides a drying effect on the nasal mucosa.

Pharmacokinetics:

Pseudoephedrine is almost completely and rapidly absorbed from the G.I.T. Pseudoephedrine is incompletely metabolised in the liver. Onset of action is within 15 to 30 minutes and has a duration of action for 3 to 4 hours. Elimination is renal and about 75% is excreted unchanged. The rate of excretion is accelerated in acidic urine.

Triprolidine is absorbed from the G.I.T. The time to peak effect is 2 to 3 hours, half life of 3 to 3.3 hours and a duration of action of 4 to 8 hours. It is metabolised in the liver and is excreted in the urine.

Indication:

Corifed Tablet is indicated for the temporary relief of nasal and sinus congestion, sneezing and rhinorrhea associated with the common cold and allergic rhinitis.

Route of administration:

Oral administration.

Recommended dose:

Adults and children over 12 years	: 1 tablet three times a day.
Children 6 – 12 years	: ½ tablet three times a day.

Contraindication:

Contraindicated in patients with a known hypersensitivity to pseudoephedrine or triprolidine. Also contraindicated in patients under treatment with monoamine oxidase inhibitors or within two weeks of stopping such treatment. CORIFED is contraindicated in patients with severe hypertension or severe coronary artery disease.

Warning and precautions:

Although pseudoephedrine has virtually no pressor effect in patients with normal blood pressure CORIFED should be used with caution in patients taking antihypertensive agents,

tricyclic antidepressants, other sympathomimetic agents, such as decongestants, appetite suppressants and amphetamine-like psychostimulants. The effects of a single dose of CORIFED on the blood pressure of these patients should be observed before recommending repeated or unsupervised treatment.

As with other sympathomimetic agents, caution should be exercised in patients with uncontrolled diabetes, hyperthyroidism, elevated intraocular pressure and prostatic enlargement.

Caution should be exercised in patients with hepatic and or renal impairment.

Interaction with other medicaments:

Because of its pseudoephedrine content, the effect of antihypertensive agents which modify sympathetic activity may be partially reversed by CORIFED.

Concomitant use of CORIFED with sympathomimetic agents, such as decongestants, tricyclic antidepressant, appetite suppressants, and amphetamine-like psychostimulants or with monoamine oxidase inhibitors which interfere with the catabolism of sympathomimetic amines, may occasionally cause a rise in blood pressure.

The pseudoephedrine content may potentially reverse the hypotensive action of drugs which interfere with sympathetic activity, including bretylinium, bethanidine etc.

Pregnancy and lactation:

Although pseudoephedrine and triprolidine have been in widespread use for many years without apparent ill consequence, there are no specific data on their use during pregnancy. Caution should therefore be exercised by balancing the potential benefit of treatment against any hazards. It has been estimated that approximately 0.5 to 0.75% of a single dose of pseudoephedrine and approximately 0.06 to 0.2% of a single dose of triprolidine ingested by a mother will be excreted in the breast milk over 24 hours, but the effect of this on breast-fed infants is not known.

Side effects:

In some patients pseudoephedrine may occasionally cause insomnia. Rarely, sleep disturbances and hallucination have been reported. Triprolidine may cause drowsiness and patients should not drive a vehicle or operate machinery until they have determined their own responses. In some patients, the drowsiness induced by antihistamines may be potentiated by alcohol or other central sedatives. Fixed drug eruption due to pseudoephedrine, taking the form of erythematous nummular patches, and lichenoid skin eruption due to triprolidine, have been reported but both these reactions should be regarded as rare events.

Urinary retention may occasionally occur in male patients in whose prostatic enlargement could be an important predisposing factor.

Overdosage:

Symptoms: May include drowsiness, lethargy, dizziness, ataxia, nystagmus, weakness, hypotonicity, respiratory depression, dryness of the skin and mucous membranes, tachycardia, hypertension, hyperactivity, hyperpyrexia, irritability, visual and auditory hallucinations, convulsions, difficulty with micturition, nausea and vomiting.

Treatment: Measures should be taken to maintain and support respiration and control convulsions. Gastric lavages should be performed. Catheterisation of the bladder may be necessary. If desired, the elimination of pseudoephedrine can be accelerated by acid diuresis or by dialysis.

Effects on Ability to Drive and Use Machine:

May cause drowsiness and impair performance in tests of auditory vigilance. Patients should not drive or operate machinery until they have determined their own response.

Storage:

Store below 30°C.

Presentation:

10 tablets in a PVDC/Aluminium blister.

1, 2, 3, 5, 10, 25, 50 or 100 blisters in a carton box.

Manufactured by:

Noripharma Sdn. Bhd., 200701034604 (792633-A)

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5 ½ Mile off Jalan Meru,

41050 Klang, Selangor,

MALAYSIA.

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